

CHARLOTTE HUNGERFORD HOSPITAL

Outpatient Primary Care - Acute Visit Record

ACUTE ILLNESS VISIT RECORD

Patient	DOB	Visit Date	Provider	Visit Type
Turner, Megan E.	06/06/1952	January 13, 2026	Steven Alford, MD	Acute Illness

CHIEF COMPLAINT

Worsening cough, increased sputum production (yellow-green), shortness of breath at rest (not typical baseline), low-grade fever. Symptoms began January 8 as upper respiratory infection; progressed by January 11.

VITAL SIGNS

BP	HR	Temp	RR	O2 Sat (room air)	O2 Sat on 1L O2
148/90 mmHg	82 bpm	99.8 F	20/min	88%	93%

ASSESSMENT

- 1. Acute exacerbation of COPD, moderate severity, precipitated by acute bacterial bronchitis
- 2. Acute bronchitis, bacterial (ICD-10 J20.9)
- 3. Hypertension, transiently elevated (stress response, not requiring adjustment)

TREATMENT / PRESCRIPTIONS

Medication	Directions	Duration	Pharmacy
Prednisone 40 mg tablets	1 tablet daily for 5 days (40mg), then 20mg daily for 5 days	10 days	CVS Thomaston
Amoxicillin-Clavulanate 875mg/125mg	1 tablet twice daily with food	7 days	CVS Thomaston
Albuterol (existing Rx)	2 puffs every 4-6 hours as needed	Until resolved	Existing supply

DISCHARGE INSTRUCTIONS

- 1. Rest. Increase fluids. Take all antibiotics as directed, even if feeling better.
- 2. Use rescue inhaler as directed above. Continue Spiriva once daily in the morning.
- 3. Continue nocturnal oxygen at 1L. Do not discontinue.
- 4. Return to ED or call 911 if: O2 saturation falls below 88% on home pulse oximeter, severe shortness of breath, chest pain, high fever above 102 F.
- 5. Follow up with Dr. Alford in 3 weeks (February 4, 2026). Dr. Chowdhury notified.
- 6. David Turner (860-555-0118) called and informed of visit and instructions.

Attending: Steven Alford, MD | Visit Date: January 13, 2026